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Name: عائشه سيد اسماعيل ابراهيم National ID: 27704062200684	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



الى من يهمه الامر / مستشفى جوستاف روسي

تحية طيبة وبعد،،،،

بالإشارة الي قانون الهيئة العامة للتأمين الصحي الشامل رقم 2 لسنة 2018 ، والذي يتضمن
بوجوب خضوع المقيمين بمحافظات التطبيق اجباريا بالمنظومة وفقا لمحل الإقامة
ببطاقة الرقم القومي داخل احدي محافظات التطبيق بورسعيد- الاقصر-الاسماعيلية -
جنوب سيناء - السويس -اسوان

برجاء التفطن بالعلم بأنه قد تم فحص بيانات
المواطنة / عالشه سيد اسماعيل ابراهيم وتحمل بطاقة الرقم القومي /
27704062200684

وبناء علي ما سبق في ان المستفيد المذكور من غير الخاضعين لمنظومة التأمين الصحي
الشامل نظرا لان محل الإقامة ببطاقة الرقم القومي محافظة بني سويف خارج نطاق
محافظات تطبيق منظومة التأمين الصحي الشامل

وتفضلوا بقبول فائق التقدير والاحترام،،،

رئيس الإدارة المركزية لشئون المستفيدين

تحرير الي : 2026/04/9
ن.ع الإدارة المركزية لشئون المستفيدين





Name: عائشه سيد اسماعيل ابراهيم		GUSTAVE ROUSSY	
MR. : 3057			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 49-year-old premenopausal individual with a history of right-sided breast cancer treated with breast-conserving surgery and radiotherapy on 8/7/2025. Pathology at that time was T2 N1, grade 3/16, estrogen-receptor score 8/8, progesterone-receptor score 6/8, and HER2 3+. Adjuvant therapy included four cycles of AC and 12 cycles of taxol, followed by ongoing tamoxifen and subcutaneous trastuzumab. A transvaginal ultrasound performed on 6/9/2026 demonstrated an increased endometrial thickness of 15 mm, prompting dilation and curettage; subsequent laboratory testing showed postmenopausal levels of estradiol and follicle-stimulating hormone. The biopsy and biomarker profile confirmed a hormone-receptor-positive, HER2-positive tumor (ER 8/8, PR 6/8, HER2 3+). Imaging revealed the previously noted right-breast lesion and the endometrial finding, while metastatic work-up was not detailed beyond the local assessment. The patient's disease is considered high-risk due to node-positive status with three positive lymph nodes. The referring team recommended shifting endocrine therapy to an aromatase inhibitor given the high-risk node-positive disease, with monitoring of estradiol levels for three consecutive months and addition of goserelin (Zoladex) if levels rise. The MDT concurred and added zoledronic acid (Zometa) every six months, contingent upon a dental assessment.			
تحريراً في ,			
		التاريخ : 23-09-2026	الوقت :
		الطبيب المعالج :	
يعتمد ,			

MDT FORM

22/ 7 / 2025 G

Name	عائشة محمد اسامه	MRN	11.4 / Co.
Age	48 yrs	ID	677.0.200.218
Menopausal status	Regular	Phone	01122296727
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	M → Fr S → Rt BCS + ALND	REASON OF MDT	Bst - OP.

Clinical Data

BX: IDC, GII

ER: 8/8 PR: 6/8 HER 2: +3 Ki67:

Sonomammography:

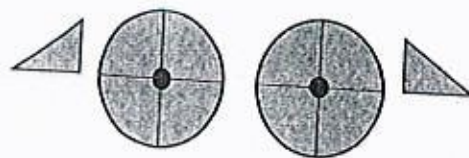
PT underwent Rt BCS + ALND

Mets Workup: GRI-E.

→ PT didn't receive recommended Treatment.

R

L



Recommendations:

Specimen → IDC GII. (PT2Ma)
Free margin.
Lm. 3/16.

+ve Perineal + lymphovascular embolus.

Dr. Hoss
20/7/2025

MDT OUTCOME & PLAN

① Adj - AC x 4

- Taxol weekly + Herceptin 1 year

② PORT

③ Tamoxifen x 5 years

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Hoss	Dr. Hoss				
	22/7/25				

HISTOCARE LAB

د. أمال أحمد هاردي

Dr. Amal Hareedy

مستشفى الباطنية والسرطان
الجامعة الأردنية - عمان

PATIENT'S NAME: **عبدالله بن اسماعيل**
AGE: 48Y
SEX: M
REFERRED BY: **PROF. DR. ASHRAF SHAWKY**
CLINICAL DIAGNOSIS: **Right Breast Cancer**
NATURE OF THE SPECIMEN: **WLE & ALNS**
SPECIMEN OBTAINED AT: **8/7/2023**

SPECIMEN REPORTED AT: **15/7/2023**

PATHOLOGY REPORT

GROSS: Two specimens were received: 1- WLE formed of infolded irregular breast tissue measured 13x7x5cm, partially covered by skin ellipse 6x3cm. Serial sections showed an irregular firm to hard grayish white mass measured 2.2x1.5cm, (totally submitted) 2- Axillary fat was 10x8cm, including 16 lymph nodes, the largest was 2x1.5cm.

MICROSCOPY: Sections examined from the mass revealed malignant neoplastic growth composed of invasive clusters, tubules & cords of moderately anaplastic ductal epithelial cells, 10% in situ component supported by fair desmoplastic stroma with lymphocytic infiltrate (peritumoral 10%). Lymphatic tumor emboli & perineural invasion were seen. The resection margins & skin were free. The lymph nodes showed positive metastatic deposits in 3/16 with extracapsular extension.

DIAGNOSIS: RIGHT BREAST CANCER; WLE & ALNS:

- **INVASIVE DUCT CARCINOMA GRADE 2, NST, 10% DCIS, POSITIVE PERINEURAL INVASION & LYMPHATIC TUMOR EMBOLI.**
- **FREE RESECTION MARGINS & SKIN.**
- **POSITIVE 3/16 LYMPH NODES WITH EXTRACAPSULAR EXTENSION.**
- **STAGE: pT2 N1a (MAXIMUM TUMOR DIMENSION 2.2cm).**

H.B: ER, PR, Her2-neu and Ki67 Immunostains are available on request

NO: Four slides enclosed
Thank you for reference
SIGNATURE

DR. AMAL HAREEDY, MD

معمل مستوكيز للتحاليل الباثولوجية والخلوية 5 من مراتب - عمارة صينيليك معمر النور الثاني - وحدة 5 ت
01096466110

Patient Name : الأستاذة / د. منة عبد الحليم

Patient No. : 27704062200684

Path No. : SR-879/25

Sex : Female

Age : 48 Y

Referring Doc :

Request No. : 13003182

Request Date : 29/06/2025

Verification Date : 30/06/2025

Financial Category : جوفات روسي - طبقة الدولة

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass

Nature of specimen

Block & slide revision

Gross examination

One block labelled by (3672/25) & three immuno-stained slides were received

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- * DCIS: Focal, solid, low grade
- * Angiolymphatic invasion: Positive
- * Perineural invasion: Absent
- * IPT review slides :
- * ER: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 20 % tumor cells (Allred score 6/8)
- * HER2: Strong complete membrane reaction in > 10 % of tumor cells (score 3+)

Diagnosis

Right breast mass, block revision

Invasive duct carcinoma, Grade 2

IPT revision:

- * ER: Positive (score 8/8).

Verified by :

Dr. Heba Hasan

Printing Date : 20-07-2025 10:00:17

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الطابق الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Al-Kharrat - Cairo, Egypt

Patient Name : الأمل / هبة / د. وردة / د. محمد / د. أحمد / د. محمد / د. أحمد

Patient No. : 27704062200684

Path No. : SR-879/25

Sex : Female

Age : 48 Y

Referring Doc :

Request No. : 13003482

Request Date : 29/06/2025

Verification Date : 30/06/2025

Financial Category : الدولة - تأمين الدولة

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

* PR: Positive(score 6/8).

* HER2: Positive(score 3+).

Signature

Dr. Walaa Ibrahim Ghoneim

Dr. Heba Hassan Adly

Verified by :

Dr. Heba Hasan

Printing Date : 20/07/2025 10:00:17

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



طبعة رقم (4) - الطور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mir El Qadima - Cairo, Egypt

BQHEYA

Imaging Systems

PATIENT NAME: ALSH SAYED ISMAIL IBRAHIM
GENDER: Female
DATE: 18-Jun-2025 12:50 PM

PATIENT ID: 2620142509
AGE: 46 y
ORDER: 3416935 3416936

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B)
- There is an increase in the size of the previously noted mass like focal asymmetry located in the deep aspect of LQ; it currently appears irregular and isodense with a spiculated margin. Complementary ultrasound revealed an irregular hypoechoic spiculated mass that is peripherally located in zone C at 5:00, 4mm deep to the skin measuring 20x13x17mm in its respective TSXAPXCO diameters.
- No left breast spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no left breast solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Axillary lymph nodes (around 3) demonstrating eccentric hila and asymmetrical cortical thickness reaching 4.6mm are noted in the right side.
- Left axillary lymph nodes showing echogenic hila and average cortical thickness reaching 2mm.

OPINION:

Right breast: malignant mass, BIRADS 5 for histopathological correlation

Left breast: nodules, BIRADS 2

RECOMMENDATION:

Referral to clinic

Report



Name: Aisha Sayed Esmail

Date: 30/6 /2025

Referring physician: Prof. Ashraf Shawkey ID: 30196819

Whole Body 18F-FDG PET/CT

History and clinical findings:

Right breast mass lesion

Reason for the study: *Detection of FDG avid lesions.*

Technique:

Radio-pharmaceutical: F-18-FDG, intra-venous.

Imaging sequence:

- * *Approximately 60 minutes following radiopharmaceutical administration, PET-CT was acquired from mid-thigh to the skull vault with the arms extended above the head.*
- * *Qualitative and semi-quantitative assessment of the PET images was done.*
- * *Diagnostic CT was acquired in addition to the purpose of anatomic localization and attenuation correction.*
- * *Data was processed and displayed according to departmental protocol.*

FINDINGS:

A. Brain:

- Normal diffuse brain activity with no hypermetabolic brain lesions.
- No focal cerebral area of abnormal density seen.
- Preserved normal gray-white matter interface.



- No extra-axial or intra-cerebral areas of fresh blood densities.
- Normal size and configuration of the ventricular system.
- No midline shift.
- Unremarkable posterior fossa structures

B. Neck:

- Small bilateral deep cervical LNS
- Prominent bilateral palatine tonsils
- Normal appearance and physiologic FDG uptake of submandibular and parotid salivary glands.
- Normal appearance of the neck muscles & vessels.

C. Chest:

- Deep lower central right breast small lesion is seen, measures about 1.8 x 1.1 cm with SUVmax~5.3 along its max axial dimensions associated with prominent right axillary LNS, largest measures 1.3x0.9 cm of SUVmax~2.4 for sonomammographic assessment
- Clear both lungs.
- No sizable mediastinal LNS
- No evidence of pleural effusion, pleural thickening or masses seen.
- No pericardial effusion, thickening or masses detected.
- Normal CT appearance and physiological FDG uptake of thoracic aorta and the great vessels.
- Normal CT appearance and physiological FDG uptake of the chest wall.

D. Abdomen and Pelvis:

- The liver is of average size with NO FDG avid focal lesions detected. Few small tiny hypodense lesions of low attenuation seen at segment III, VIII, IVb, largest measures 5 mm, tiny right lobe segment VI small fat containing lesion, measures about 4 mm



- Right iliac region clarified nodule seen along the surface of the cecum.
- The spleen is of average size with NO FDG avid focal lesions detected.
- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder.
- No evidence of ascites or abdominal collections noted.
- Bilateral adnexal cysts, largest at right, measures about 3.5x3.2 cm
- Bulky uterus with internal endometrial hypodensity seen within for U/S assessment

E. Musculoskeletal:

- Normal FDG activity is seen over the axial skeleton.
- No focal active blastic or lytic bone lesions detected.

Conclusion:

- Metabolically active right breast nodule ? site of primary associated with FDG avid ipsilateral axillary LNs ? metastatic, for sonomammographic assessment.

Thanks for referral,

Dr. Eman Nasr, MD
Radiology Consultant

Dr. Esraa Roshdy, MD
Nuclear Medicine Consultant

Echocardiography Report

Name: ASIA SAYED
Date: Monday, July 07, 2025.
Age : 48Yrs.

M-Mode Measurements:

LVEDD	4.4	N (3.5-5.6 cm)	Aorta	2.9	N (Up to 3.7cm)
LVESD	2.9	N (2.5-4.5 cm)	LA	3.0	N (Up to 4.0 cm)
IVS	0.9	N (0.7-1.2 cm)	FS	33%	N (>25%)
PW	0.9	N (0.7-1.2 cm)	EF	62%	N (>54%)

Comment:

- Left ventricle shows normal internal dimensions and normal global systolic function, EF= 62 % by M-mode & about the same range by 2D eyeballing.
- Normal LV wall thickness.
- Impaired LV diastolic function grade 1
- No resting SWMA could be detected at rest.
- Normal LA diameter.
- Normal aortic root dimensions.
- Normal MV structure and motion with mild MR.
- Normal AV with max. systolic gradient across= 5 mmHg, no AR.
- Normal TV structure with trivial TR max SPG=31mmHg, normal estimated RVSP
- Normal PV structure and motion, trivial PR.
- Normal right side dimensions with good RV systolic function.
- Normal pericardium, no pericardial effusion.
- No intra cardiac masses or thrombi detected.
- Intact interventricular and interatrial septae.

Much obliged,
Dr. Elham E. Elhawary



جمهورية مصر العربية

بطاقة تحقيق الشخصية

عائشه

سيد اسماعيل ابراهيم

كوم ابوراضى

مركز الواسطى - بنى سويف



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LU0976081



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ربه منزل



أنثى مسلمة متزوجة
محمد صلاح عويس عثمان

البطاقة سارية حتى ٢٠٣٣/٠٦/٠٥

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